

Patient Assessment

A. Steps When arriving on-scene (Scene Size up):

1. **I'm #1** (Make sure the scene is safe for you to enter!!)
2. **What happened to you?** (Look for MOI and NOI)
3. **None on me** (PPE and BSI) (Gloves!!)
4. **Is there anymore?** (How many patients?)
5. **Keep 'em alive!** (Do you need other resources? Patrollers?)

B. Steps for a Primary Assessment (ABCDE, LOR) (C comes first if there is bleeding or in Cardiac arrest!!):

1. Introduce yourself (Consent!)
 - a) "Hi, my name is _____, I'm with ski patrol, Can I help you?"
2. LOR - If I can talk/communicate with them - GREAT!
 - a) **AVPU** - **Alert, Verbal** (opens eyes to voice), **Pain** (stimuli), **Unresponsive**

3. If there is bleeding (STOP and FIX!!)

- a) Apply pressure, use pressure dressing, tourniquet, or pack it!

4. Assess ABCDE (STOP and FIX):

a) Airway

- (1) **Conscious/Responsive:** Ask if there is anything in their mouth? Ask them to open their mouth.
- (2) **Unresponsive/Unconscious:**

A - Unresponsive	Trauma	No Trauma
Snoring = Dry (Tongue)	Jaw Thrust	Head tilt - Chin lift
Gurgling = Wet (Liquid)	Log Roll & Make Gutter	Log roll & turn head & gutter

b) Breathing

- (1) **Look** - for rising and falling of chest
- (2) **Listen** for sound of in and out
- (3) **Feel** Chest wall rise and fall

c) Circulation (Blood Sweep! Feeling wet or sticky?)

- (1) **Feel** for Pulse (No numbers needed right now!)
- (2) **Assess** skin color
 - (a) pink/warm/dry/pale/cool/clamy/blue/cold/wet/red/hot
- (3) **Capillary** refill (2 sec)

d) Disability or Duh brain

- (1) **Ask** the following questions: →

e) Environment

- (1) How long have they been outside
- (2) Move them off the snow ASAP

5. Obtain their chief complaint (their words) & Expose if needed

Does the Patient have a Spinal/Head Injury?	
What's your name?	A+O x4
Where are you?	A+O x3
What day is it? Or time?	A+O x2
What happened?	
If can't answer first two questions: - Ask if there is any discomfort in the back/neck/head. - Suspect head/spinal - Tell them to keep their head as still as possible - If yes to pain, palpate upper neck to big bump	

C. Steps for a Secondary Assessment:

1. Head to Toe (Anything hurt?) (Penguin paddles. No creepy hands.)

1. Palpate head
2. Look in Ears
3. Check face for signs of trauma (No need to palpate)
4. Check neck (No palpating)
5. Crawl fingers down clavicles
 - a) One at a time!
6. Palpate shoulders
 - a) One at a time
7. Palpate Sternum with fist
8. Check upper chest
 - a) pushing on both sides; Have them take a deep breath
9. Check lower chest (Ribs)
 - a) pushing on both sides; have them take a deep breath
10. Palpate four quadrants of abdomen
 - a) Ask patient to point to their belly button, or pull up shirt
 - b) Roll the hands (boat rocking motion)
11. Push on both sides to check pelvis
12. Check each leg (lower and upper) and bend each knee
 - a) Hands at staggered position, pushing against bone
13. Check feet
 - a) Pulses
 - b) Palpate, have them push down and up, and if they can feel you touching their feet
14. Check arms, bending elbows (same as legs)
15. Then place hands on chest, and check pulses (radial)
16. Ask them to squeeze your hands, wiggle them
17. Touch a finger on each hand and ask them which one you are touching.
18. Then Log roll patient, resting them on you, and crawl your fingers down their spine to waist
19. Push on either side of waist-area
20. Then roll to back again

2. Vital Signs:

- a. Time - Military
- b. LOR (A+O) and **Cap Refill**
- c. BP - Blood pressure (mm Hg): 120/80
- d. SPO2 - 94%-100%
- e. Skin - **SCTM** - **Skin, color, temp, moisture**
- f. Pupils - **PERRL** - **Pupils are Equal, Round, and Reactive to Light**
- g. HR - Heart Rate (bpm): 60-100 bpm
 - i. Pulse (Radial or Carotid)
 1. Rate - bpm
 2. Quality - strong, weak, pulse
 3. Rhythm - steady beats
- h. RR - Respiration Rate (bpm): 12-20
 - i. Breathing
 1. Rate - breaths per minute
 2. Quality - gasps, deep, etc.?
 3. Rhythm - (rise/fall) Of one breath

D. History Taking (SAMPLE and OPQRST):

SAMPLE		OPQRST	
S	Signs/Symptoms (How Patient feels)	O	Onset (When Symptoms began)
A	Allergies (Medications)	P	Provocation & Palliation (What makes it better?)
M	Medications Taken/Drugs/Alcohol	Q	Quality (Description of injury/illness)
P	Past Medical History (Recent in Hospital?)	R	Radiation (Does the pain travel?)
L	Last ins and outs (Bathroom)	S	Severity (Scale 1-10) (What's their 10?)
E	Events leading up to incident	T	Time (How long?)

E. Reassessment:

1. Reassess Vitals every 15 minutes if stable. Do every 5 minutes for unstable patients.
2. Repeat ABCDs, etc.